

CANONICAL LESSON

V7_B_N15 - National Health Intelligence Capstone

Decision-centred foundations for accountable sector intelligence

Production Unit	PU-B07-C35
Notebook	V7_B_N15
Status	Controlled educational draft v0.1
Data boundary	Synthetic instructional data only

Central decision: Integrate governed health evidence, surveillance, capacity, equity, evaluation, referrals, and accountability into a national decision product.

Learning outcomes

Why this matters

Integration without governance can centralize error and weaken clinical and public accountability.

The analytical workflow is useful only when evidence, uncertainty, authority, and action are connected explicitly. The model supports professional judgement; it does not replace accountable institutional authority.

Worked cross-sector example

A national brief connects indicators, risks, resources, uncertainty, and authorized actions.

Boundary: The example supports learning and review. It does not constitute an official statistic, eligibility determination, enforcement action, clinical judgement, or operational authorization.

Practice and reflection

- 1.
- Separate surveillance from diagnosis.
- 2.
- Design an incident handoff.
- 3.

- Audit equity under scarce capacity.

Answer key

- 1.
- Surveillance detects population signals for verification; diagnosis is a licensed individual clinical act.
- 2.
- Record evidence, uncertainty, urgency, owner, timestamp, command acceptance, action, override, and closure review.
- 3.
- Examine need, access barriers, false errors, waiting time, benefit, harm, and appeal across groups without ranking human worth.

Operational readiness checklist

- Purpose and decision owner are named.
- Evidence definitions, provenance, and lawful access are documented.
- Uncertainty and abstention are visible.
- Human review, escalation, monitoring, incident response, and retirement are assigned.
- Equity, accessibility, security, and sector-specific approval are complete.